



Document Management System



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Title: Confidentiality Policy

1. Introduction:

Creating a trusting environment by respecting patient privacy encourages the patient to seek medical advice and to be as honest as possible during the course of a health care visit. It may also increase the patient's willingness and desire to seek care. For conditions that carry stigma, such as reproductive, sexual, public health, and psychiatric health concerns, confidentiality assures that private information will not be disclosed to family or employers without patient consent. Lack of mutual trust will diminish or even destroy physician-patient relationship. Appropriate information sharing however sensitive it may be, is essential for safe and effective care, and in turn reflects on the quality of care perceived.

Electronic medical records can pose challenges to confidentiality. In accordance with quality care and patient's rights, policies are required to protect the privacy of patients' electronic information, including procedures for computer access and security which will be outlined in this document.

Ensuring patient confidentiality is part of the patient bill of rights. We as HealthCare Provider (HCPs) have a responsibility to protect the rights of patients in every way possible. This policy is a reminder to the Hippocratic medical oath taken by HCPs, which must be adhered to. Trust is built on confidentiality, other industries have taken serious steps to secure and protect their clients trust. We in health need to be more vigilant in this failing which there could be serious ramifications both socially and legally.

2. Purpose:

This policy delineates the obligation of health care provider (HCP) in maintaining the confidentiality of patient's and prohibit disclosing information about the patient's case to others without permission and encourages the providers and health care systems to take precautions to ensure that only AUTHORIZED access occurs regarding patient information. Appropriate care often requires that information about patients be discussed among members of a health care team; all team members having authorized access to confidential information assume the duty of protecting that information from others who do not have access. "There are two over reaching principles to confidentiality: information should be (1) reserved exclusively for intended purposes, and (2) used only by authorized individuals" (Marr, 2010, pp.44)

3. Procedure:

3.1 Mental Checklist for ensuring confidentiality.

- A confidentiality self-checklist is a means for all HCPs to go through in their mind before divulging any medical information:
- Who wants to know?
- What do they want to know?
- Are they entitled to know?
- Has the patient given consent?

Here the HCP must ask themselves, who is requesting this information. HCP has a natural inclination to divulge information to family members, like spouse or son. The requirements for making an exception to confidentiality may not be met. If there is no consent or permission from the patient to share information with family member, it is generally not ethically justifiable to do so, even if the news is good. Even in cases where the spouse is at specific risk of harm directly related to the diagnosis, it remains the patient's (and sometimes local public health officers'), rather than the physician's, obligation to inform the spouse. E.g. husband diagnosed with HIV: as a HCP you will advise the husband that he is to inform his wife, and it is his obligation to do so. Also you will inform the husband that you can be present to help him in divulging the information and that on the next visit you require the wife to be present and fully informed. Only if and when the husband refuses to divulge the information, a HCP, for the protection of another individual has the authority to bypass him and inform his wife. Another common scenario is phone calls asking for patient information or results. Again here, the only person with the right of this information is the patient unless he has consented for another individual to follow-up his results. In some institutions a password is used to identify the consented individual or even the patient such as "Apple". As members of bioethics committee we highly recommend that such a system is initiated to protect patient rights to confidentiality.

3.3 What do they want to know?

Here the HCP must assess what the enquirer wants to know about the patient. Just because they have passed the first checklist doesn't mean you can divulge all the patients' information. If not, then you have an obligation to protect this secret even from your own colleague.

3.4 Are they entitled to know?

Here a HCP reassesses what information if any the requester is entitled to know. This applies even in referrals, e.g. a patient being referred for colonoscopy, the gastroenterologist will not benefit in knowing the patient is divorced nor has social problems with his family. Thus this information will remain confidential. This question is also a double check and is related to the first question of who wants to know. Just because a doctor is calling to ask about your patient doesn't mean he is entitled to know. You as a HCP will have to confirm whether the patient has been referred to the doctor or not. If no referral is in the system the caller (even if a HCP) has no right to know and is not entitled.

3.5 Has the patient given consent?

Here we go back to the principles of confidentiality, which state that the information is privileged only to the patient and who the patient consents to share the information with. In the OPD setting there are two approaches to consent, one is implied consent where the patient by the fact that they walked into the doctors room with another individual are consenting to the HCP to speak in-front of them about their medical information. The second approach is the HCP asking the patient if this individual needs to be present during discussions. In the bio ethics committee we prefer the second option and the same to be documented in the electronic health care system AL-SHIFA.

3.6 Avoid breaches and inappropriate disclosures:

Unintended disclosures may occur in a variety of ways. For example, when pressed for time, providers may be tempted to discuss a patient in the elevator or other public place, but maintaining privacy may not be possible in these circumstances. Similarly, extra copies of handouts from teaching conferences that contain identifiable patient information should be removed at the conclusion of the session in order to protect patient privacy. Any identifiable patient information should either be encrypted or should not be removed from the security of the health care institution. In addition leaving AL-SHIFA system open and visible to others leaves patient information vulnerable. The patient's right to privacy is violated when lapses of this kind occur.

3.7 Electronic access of information:

AL-SHIFA is the name of the Royal Hospital information system, which unfortunately, if not used well, can lead to breach of confidentiality. All HCPs should understand that the access they have been given is a privilege and with that privilege comes great responsibility and with responsibility, accountability. Lapses in patient confidentiality can easily happen in an unmonitored system. HCPs access patient results for friends and relatives as help, not knowing that by doing so they are breaching confidentiality. Also accessing patients' files if a patient isn't in their direct care or hasn't been referred is a major breach of confidentiality. As Bioethics committee we recommend that a clear circular warning HCPs not to access patient information unless they are entitled, and to instate a random audit to evaluate adherence to this circular. This action may curb and stop these breaches completely.

3.8 When can confidentiality be breached?

- Concern for the safety of other specific persons:

Access to medical information and records by third parties is legally restricted. Yet, at the same time, clinicians have a duty to protect identifiable individuals from any serious, credible threat of harm if they have information that could prevent the harm. The determining factor is whether there is good reason to believe specific individuals (or groups) are placed in serious danger depending on the medical information at hand. An example is homicidal ideation, when the patient shares a specific plan with a physician or psychotherapist to harm a particular individual. Ethically, most would agree that a duty to warn an innocent victim of imminent harm overrides a duty to confidentiality, but these cases are rare and judgment calls of this sort are highly subjective. Hence, the duty to maintain confidentiality is critical, but may be overridden in rare and specific circumstances.

- Legal requirements to report certain conditions or circumstances: The law requires the reporting of certain communicable/infectious diseases to the public health authorities. In these cases, the duty to protect public health outweighs the duty to maintain a patient's confidence. From a legal perspective, the State has an interest in protecting public health that outweighs individual liberties in certain cases. For example, reporting

governing exceptions to patient confidentiality.

3.9 Test for Breach of Confidentiality

In situations where you believe an ethical or legal exception to confidentiality exists, ask yourself the following question: will lack of this specific patient information put another person or group you can identify at a high risk of serious harm? If the answer to this question is no, it is unlikely that an exception to confidentiality is ethically (or legally) warranted. The permissibility of breaching confidentiality depends on the details of each case. If a breach is being contemplated, it is advisable to seek legal advice or contact the bioethical committee before disclosure.

3.10 What are the confidentiality standards regarding adolescents?

In many countries adolescents may seek treatment without the permission of their parents for certain conditions, such as treatment for pregnancy, sexually transmitted infections, mental health concerns, and substance abuse. Familiarize yourself with country and local laws, as well as institutional policies, regarding adolescents and healthcare.

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[Back](#)

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